

REVENUE-CYCLE RECOVERY · REFERENCE

Recovering Patient A/R

Early-out, insurance follow-up, and bad-debt recovery that complements your revenue cycle — where the billing process runs out

This is a paper about the last stage of the revenue cycle, not the first. Coding, claim submission and payer adjudication are your billing team's and your RCM platform's job; what happens to the balances that survive that process — unresolved patient responsibility and denied, underpaid or stalled claims — is a recovery problem, and it is where most net revenue quietly leaks. For a health system, the biggest lever on that leakage is timing: a balance worked while it is fresh resolves at a far higher rate than the same balance a year later. This reference explains why patient A/R is so hard to collect, how recovery falls with age, and how an early-out and follow-up partner recovers what billing alone leaves on the table.

Executive summary

- This is **recovery, not billing** — we work patient balances, denied/underpaid/pending claims and bad debt *after* billing has run its course; we don't code claims, bill primary payers or replace your RCM platform.
- Patient collections are now the **primary revenue concern for 71% of providers**, and average collection times already exceed **30 days** before an account ever reaches recovery.
- Recovery is driven by timing: **a patient balance worked at 60 days resolves at a far higher rate than the same balance at 300 days**. Early-out balances (30–90 days) and unworked claims are where the recovery is.
- Patient responsibility keeps rising — the average single-coverage deductible reached **\$1,886 in 2025 (up 43% over ten years)**, with ~34% of covered workers in \$2,000+ plans — so more net revenue now depends on collecting from the patient.

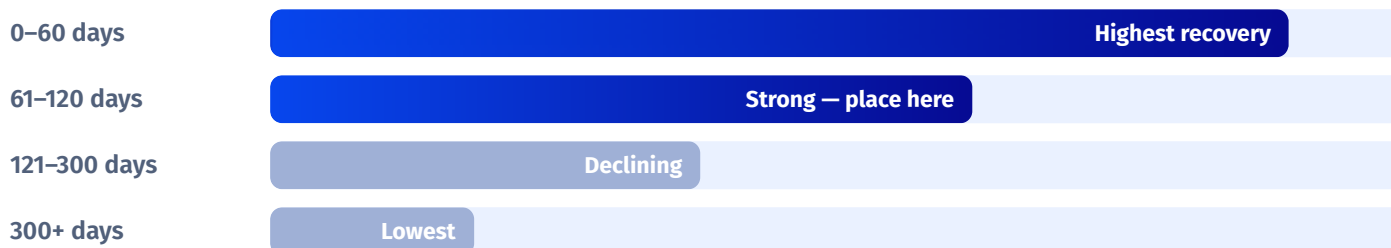
1. Where net revenue leaks — after billing

By the time a balance is a recovery problem, the revenue cycle has already done its work: the claim was coded, submitted and adjudicated, statements went out, and the account still did not resolve. Two buckets remain, and both leak. The first is **patient responsibility** — the copays, deductibles and coinsurance left after adjudication — which has grown sharply as high-deductible plans have spread (the average single-coverage deductible now sits at \$1,886, up 43% over a decade, with roughly a third of covered workers in \$2,000-plus plans). Patients are far harder to collect from than payers, and this is now the share of revenue that grows every year. The second bucket is **claims that stalled** — denied, underpaid or pending — which sit against payer resolution and appeal deadlines

that quietly expire. Neither bucket collects itself, and neither is a billing problem anymore; both are recovery work.

2. Recovery falls as the balance ages

The age of a balance is the strongest predictor of whether it is ever recovered. Contact information goes stale, the patient disengages, the balance loses urgency against newer household bills, and on any claim-side balance the payer's timely-filing and appeal windows close for good. The pattern below is the recovery-by-age curve every patient-A/R portfolio follows:



Illustrative and relative — actual recovery varies by payer mix, balance size, documentation quality and contact-data accuracy. As SWRS puts it, a patient balance worked at 60 days resolves at a far higher rate than the same balance at 300 days. We give an honest, age-based read on a representative sample before any accounts are worked.

3. Early-out: working balances before they harden

The highest-leverage change most revenue-cycle teams make is to work balances earlier, as an extension of the business office, rather than waiting until they age into bad debt. An early-out program contacts patients in the 30–90 day window while the balance is fresh and the account is still accurate: a documented, HIPAA-compliant, FDCPA-governed sequence of statements, calls and payment-plan options that validates contact data, resolves small self-pay balances before they harden, and sets up workable, interest-free payment plans — with the online payment options a majority of patients now prefer. Fewer accounts reach bad debt, more resolve at full or near-full value, and cash arrives weeks sooner. Early-out does not replace bad-debt placement; it runs ahead of it, so the two together capture far more than a single late hand-off.

4. Insurance follow-up: recovering the claim side

Not every unresolved balance is the patient's. Denied, underpaid and pending claims represent net revenue your organization has earned but not been paid, and they sit against hard deadlines. Effective recovery here means tracking each payer's filing and appeal windows, following the specific resolution path each claim requires, and escalating before the recovery window closes — work that an internal team, already stretched, often cannot chase claim-by-claim at volume. Pursued in time, a meaningful share of denied and underpaid claims is recoverable; left to age, they convert to write-offs the same way patient balances do.

5. Make recovery a standing rule

Timing only pays if it is systematic. The teams that recover the most run a rule rather than deciding account by account: current self-pay balances go to early-out; unworked or stalled claims are followed up on a fixed cadence; and any account still unresolved at 90–120 days is placed automatically, wired to the billing system's age triggers so nothing sits. The aged backlog is placed in parallel as a one-time sweep — on contingency there is no cost to trying, and even partial recovery on written-off balances is money already conceded. Measured by placement age on your own portfolio, the gap between fresh and aged recovery makes the case for the rule.

How Southwest Recovery fits your revenue cycle

We are the recovery layer, not a billing or RCM replacement — we resolve what the billing process could not. Working under a Business Associate Agreement with full HIPAA and FDCPA / Regulation F discipline, we take patient-A/R and unworked-claim placements through a secure portal (bulk-upload an aging export; we map your columns, no re-keying), open outreach within 24–48 hours, and pursue both patient balances and insurance follow-up on a documented, compliance-first cadence reported on a live dashboard. It is pure contingency — no retainer, no setup fee, no per-account charge — so it complements your RCM platform and business office rather than competing with them, and your staff stay on patient care instead of chasing aged balances.

The net effect is a revenue cycle that stops losing recoverable dollars to the calendar. Early-out captures the balances that would otherwise age; insurance follow-up recovers the claims that stalled; and prompt bad-debt placement catches the rest before payer deadlines and write-off rules close the window — all at zero fixed cost. Timing, the one variable most under your control, finally works in your favor.

About Southwest Recovery Services

Southwest Recovery Services is a HIPAA-compliant medical and healthcare receivables recovery firm, working patient and payer balances on pure contingency since 2004 — for health systems, hospitals, physician groups, and specialty and post-acute providers. We operate under a Business Associate Agreement with every healthcare client, hold staff to ongoing HIPAA and FDCPA / Regulation F training, and furnish qualifying accounts to Equifax and Experian only where lawful (honoring the evolving state medical-debt reporting restrictions). Over two decades we have placed more than \$975M in receivables, recovered over \$76M for clients, and worked 800,000+ accounts nationwide from 12 offices in 7 states — on pure contingency, with no upfront fee, no retainer and no minimum, and a clean compliance record since 2004.

Put a number on your patient A/R

We'll review a representative sample of your patient-responsibility balances and unworked claims and give you an honest, age-based read on what is recoverable — and what earlier placement would add to net revenue.

Call **(866) 551-4684** · sdietz@swrecovery.com · swrecovery.com

Provided for general informational purposes; not legal advice. Statutes and credit-reporting rules for medical debt vary by state and change frequently — confirm current requirements with counsel. Recovery figures are illustrative and vary by portfolio. Southwest Recovery Services is HIPAA-compliant and does not collect consumer debt in California, Oregon or Washington.